

العمر(تاريخ الميلاد) : 21-03-1979 المهنة : بدون عمل

المجلس الطبي المتخصص

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Name: سره احمد صديق كفافي National ID: 27903212100724	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سريه
احمد صديق كفاقي
قرية الشوبك الشرقي
مركز الصف - الجيزة

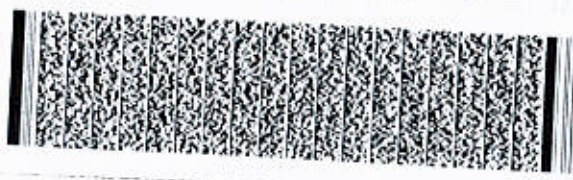
٢٧٩٠٣٢١٢١٠٠٧٢٤

KB3111968

٢٠٢٣/٠٦ (٢٧٩٠٣٢١٢١٠٠٧٢٤)



ربة منزل
أنثى
مسلمة
متزوجة
محمد محمد احمد علي سلامة
البطاقة سارية حتى ٢٠٣٠/٠٦/٢٤

100m.

15/12/2024, 10:50

code:42 -- Name_A MDT form Harmat -- Name_E MDT form Harmat

Presidential Program for Women's Health
Ministry of Health and Populationالبرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

A-patient ID:

Name: مريم احمد صديق كفاقي

Sex:

MDT form

1- Medical number:
Female

83215639

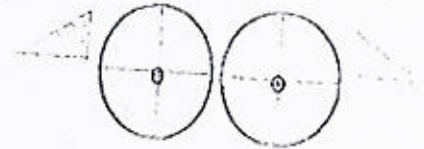
Old Med:

Date of MDT:

24-2024

2- ID number:

Patient Age: 46 Y



Family history

Family history
+ve

Phone

Phone

01198538831

BMI

BMI

Marital status

Marital status
+ve

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co morbidity
dm, htn,
CARDIACMenopausal
statusMenopausal status
+ve

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT

Date of MDT

Date of MDT

15/12/2024

Breast Imaging:

It breast mass solid lesion UOQ BIRADS 4

Clinical data

Mets Workup:

CT CAP: FREE

ECHO: EF 41 %, CARDIOMYOPATHY

BONE SCAN: FREE

BX

IOC

BX COMMENT

ER

8/8

ER COMMENT:

PR

7/8

PR COMMENT:

Her 2 Neu

3

Her 2 Neu COMMENT:

SISH

SISH COMMENT:

Ki67

20

T

3

N

1

M

0

Sonomammography

PATIENT HISTORY

RECEIVED NACT WEEKLY CARBOP AUC 2 / TAXOL 60MG/M2 X2 WEEKS

ANAPHYLAXIS, ICU ADMISSION PATIENT DIDNT DO CARDIOCONSULTATION, NON COMPLIANT

upfront Lt BCS + SLNB:

T2 LN0/17,

free margins

New echo: EF 58% // cardiac cath: normal except Lt bundle br. block

MDT OUTCOME & PLAN

or -taxol (+ solucortil in premedication & close monitoring) + herceptin 600mg S.C x 1 year

PORT

adj AI / zoladex, Zometa / 6 months.

X18ul

SIGNATURE

RADIOLOGY

DATE: 03

MDT Form

[[DOCTYPE:MDT_FORM]]

Patient Name : الأستاذة / مريمه احمد صديق كفاقي

Patient No. : 27903212100724

Birth No. : 1625/24

Sex : Female

Age : 45 Y

Referring Doc. :

Request No. : 13009003

Request Date : 18/05/2024

Verification Date : 29/05/2024

Financial Category : وزارة دار السلام - نفقة الدولة

Department : Out Patient

IHC No. :

Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).

- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).

- Ki67: 20% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma with atypical medullary features, Grade III.

IPT:

- ER: Positive (score 8/8).

- PR: Positive (score 7/8).

- Her2: Positive (score 3+).

- Ki67: Positive in 20% of tumor cells.

Comment:

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining: 1,

Verified by :

Dr.Rofayda Abdallah

Printing Date : 11/06/2024 09:41:45

Page 1 of 2

Printed By : marwa mohamed yousse



Women's Health Outreach Program

Name: Seria Ahmed Sedek

Date: 16- 5-2024

Bilateral Full field Digital Mammography

Clinical summary: 44 years left breast palpable lump for 5 month

Technique

Mediolateral oblique and CC views

High resolution ultrasound

Detailed Findings

Right breast

- Fairly dense mammary parenchyma (ACR b)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound: No cystic or solid masses seen.
- Normal skin/nipple/areolar complex.
- Non specific axillary lymph nodes detected.

Left breast

- Fairly dense mammary parenchyma (ACR b)
- UOQ ill defined dense lesion
- Ultrasound:
 - Corresponding to mammo finding shows well defined irregular hypoechoic solid lesion seen para areolar at 1 clock measures 30x21mm away from nipple by 25mm and skin by 3mm
 - Enlarged multiple lymph nodes with preserved fatty hilum at level I
 - Enlarged one pathological axillary lymph node seen at Level II measures 14x5mm

Impression:

Normal right breast, BIRADS 1

Left breast BIRADS 4 c. UOQ malignant looking solid lesion

Recommendation: Biopsy was taken at same sitting

Best regards,
Dr :Marwa M,Hassan



Echo report

Name: Date: 2/1/ 2025Age:

EF	54-74%	PWT		
LVEDD	3.5-5.7cm	AO		0.6-1.1cm
LVESD	2.2-4.0cm	LA		2.0-3.7cm
FS	25-45%	RV		1.9-4.0cm
SWT	0.6-1.1cm	AO.CUSP		0.7-2.3cm (supine)
				1.5-2.6cm

LV dimensions:

LV wall thickness:

LV diastolic function:

SWMA.

LA size

Aortic root dimension

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side size and systolic function

Intracardiac masses and thrombi

Pericardium

IVC diameter and collapsibility.

Conclusion:

D/

???, ??? Patient ID: 10110224005776 DOB: 21-Mar-1979 Date: 06-Aug-2024

atient Lastname: احمد

atient Firstname: سريه

IIS Patient ID: 10110224005776

US Patient ID: 357070

Date of Birth: 21/03/1979

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

9mTc MDP Dual Phase Bone Scan Report (06/08/2024)

Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
revealed:

No definite evidence of active osseous deposits seen all over the surveyed skeleton.
Degenerative changes over the dorso-lumbar vertebrae.

Arthritic changes are noted over both shoulder and both knee joints. >

Both kidneys are visualized.

Conclusion: -

No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Mahmoud Akram

Dr. Gamilia Talaat, MD

===== REPORT TEXT ENDE =====